

Hypopituitarism

KEYWORDS / TYPICAL PRESENTATION

- Gradual onset fatigue, amenorrhea/low libido, cold intolerance. History of pituitary surgery, radiation, or head trauma.
- **Pituitary Apoplexy: Sudden severe headache, visual changes, AMS.** A medical emergency.

DIAGNOSTIC PATHWAY

- Suspect secondary adrenal insufficiency: **Low cortisol with a low/normal ACTH.**
- Suspect secondary hypothyroidism: Low Free T4 with a low/normal TSH.

Core Principles for Hypopituitarism Management:

1. **Cortisol First:** Replace before thyroid hormone. Giving thyroid hormone before cortisol replacement can precipitate life-threatening adrenal crisis (thyroid hormone increases cortisol clearance).

AVOID: Levothyroxine before **steroids** in panhypopituitarism (can trigger adrenal crisis).

Pituitary Adenomas

KEYWORDS / TYPICAL PRESENTATION

- Headaches, **bitemporal hemianopsia**, hormonal symptoms.
- Woman with **amenorrhea + galactorrhea**: Think Prolactinoma.
- Acral enlargement, coarse facial features: Think Acromegaly.
- Empty Sella Syndrome: Incidental finding. Reassure.

DIAGNOSTIC PATHWAY

- **Imaging Test of Choice:** Brain MRI.
- **Initial lab workup for any sellar mass:** Prolactin (>200 ng/mL indicates prolactinoma), TSH/Free T4, 8 AM cortisol, IGF-1, and LH/FSH.

Core Principles for Pituitary Adenoma Management:

1. **Prolactinomas:** Treated MEDICALLY first with a dopamine agonist (Cabergoline).
2. **Other Adenomas:** Treated surgically.

PEARL: Most **Prolactinomas** (even large ones) respond to medical therapy. Surgery is rarely first-line.